

**Name** : Srinath(24Y/M)

**Date** : 9 Sep, 2026

**Test Asked** : Comprehensive Full Body Checkup With Vitamin D And B12 - 3, Mg + 2 Others

**Report Status** : Complete Report



## **6<sup>STEP</sup>** quality control to ensure 100% report accuracy



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Processed At :  
H. NO. 1-9-645, Vidyanagar,  
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Hyderabad-500044



DOCON TECHNOLOGIES PRIVATE LIMITED, Office No.208, 209, 210 Second floor,  
A wing, 'Dattani Plaza', Near East West Industrial Estate, Safed Pool, Saki Naka,  
Andheri (East), Mumbai - 400072

Patient Name

: SRINATH(24Y/M)

Referred By

: SELF

Address

: 6-7-63 RAJU COLONY HINDUSTAN AERONAUTICS HYD HYDERABAD-1

Tests Done

: Comprehensive Full Body Checkup With  
Vitamin D and B12 - 3,MG,TEST,IRON  
DEFICIENCY PROFILE ADVANCED

Report Availability Summary

**Note:** Please refer to the table below for status of your tests.

16 Ready

0 Ready with Cancellation

0 Processing

0 Cancelled in Lab

| TEST DETAILS                                               | REPORT STATUS |
|------------------------------------------------------------|---------------|
| TESTOSTERONE                                               | Ready         |
| IRON DEFICIENCY PROFILE ADVANCED                           | Ready         |
| FERRITIN                                                   | Ready         |
| TRANSFERRIN SERUM                                          | Ready         |
| IRON DEFICIENCY PROFILE                                    | Ready         |
| MAGNESIUM                                                  | Ready         |
| Comprehensive Full Body Checkup With Vitamin D and B12 - 3 | Ready         |
| FASTING BLOOD SUGAR(GLUCOSE)                               | Ready         |
| 25-OH VITAMIN D (TOTAL)                                    | Ready         |
| IRON                                                       | Ready         |
| VITAMIN B-12                                               | Ready         |
| COMPLETE URINE ANALYSIS                                    | Ready         |
| HBA PROFILE                                                | Ready         |
| HEMOGRAM - 6 PART (DIFF)                                   | Ready         |
| LIVER FUNCTION TESTS                                       | Ready         |
| KIDPRO                                                     | Ready         |
| LIPID PROFILE                                              | Ready         |
| T3-T4-USTSH                                                | Ready         |



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Patient Name : SRINATH(24Y/M)  
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Address : 6-7-63 RAJU COLONY HINDUSTAN AERONAUTICS HYD HYDERABAD-1

Tests Done : Comprehensive Full Body Checkup With  
Vitamin D and B12 - 3,MG,TEST,IRON  
DEFICIENCY PROFILE ADVANCED

Tests Outside Reference Range

**Note:** Please refer to the table below for tests outside reference range.

| Test Name                   | Observed Value    | Units | Bio. Ref. Interval. |
|-----------------------------|-------------------|-------|---------------------|
| COMPLETE URINE ANALYSIS     |                   |       |                     |
| UROBILINOGEN                | Trace(1-2 mg/dl ) | mg/dL | <=0.2               |
| DIABETES                    |                   |       |                     |
| AVERAGE BLOOD GLUCOSE (ABG) | 82                | mg/dL | 90-120              |
| LIPID                       |                   |       |                     |
| HDL / LDL RATIO             | 0.24              | Ratio | > 0.40              |
| HDL CHOLESTEROL - DIRECT    | 30                | mg/dL | 40-60               |
| LDL / HDL RATIO             | 4.2               | Ratio | 1.5-3.5             |
| LDL CHOLESTEROL - DIRECT    | 127.05            | mg/dL | < 100               |
| TC/ HDL CHOLESTEROL RATIO   | 6                 | Ratio | 3 - 5               |
| TRIG / HDL RATIO            | 5.31              | Ratio | < 3.12              |
| TRIGLYCERIDES               | 162               | mg/dL | < 150               |
| LIVER                       |                   |       |                     |
| ALBUMIN - SERUM             | 4.84              | gm/dL | 3.2-4.8             |
| VITAMIN                     |                   |       |                     |
| 25-OH VITAMIN D (TOTAL)     | 18.9              | ng/mL | 30-100              |
| VITAMIN B-12                | 153               | pg/mL | 197-771             |

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Patient Name : SRINATH(24Y/M)  
Referred By : SELF  
Address : 6-7-63 RAJU COLONY HINDUSTAN AERONAUTICS HYD  
HYDERABAD-1

Sample Collected on (SCT) : 09 Sep 2026 06:53  
Sample Received on (SRT) : 09 Sep 2026 11:14  
Report Released on (RRT) : 09 Sep 2026 13:57  
Sample Type | Barcode : EDTA Whole Blood | GP028145

| TEST NAME                                 | METHODOLOGY           | VALUE | UNITS                  | Bio. Ref. Interval. |
|-------------------------------------------|-----------------------|-------|------------------------|---------------------|
| <b>HEMOGLOBIN</b>                         | SLS-Hemoglobin Method | 15.4  | g/dL                   | 13.0-17.0           |
| Hematocrit (PCV)                          | CPH Detection         | 45.3  | %                      | 40.0-50.0           |
| Total RBC                                 | HF & EI               | 5.05  | X 10 <sup>6</sup> /μL  | 4.5-5.5             |
| Mean Corpuscular Volume (MCV)             | Calculated            | 89.7  | fL                     | 83.0-101.0          |
| Mean Corpuscular Hemoglobin (MCH)         | Calculated            | 30.5  | pg                     | 27.0-32.0           |
| Mean Corp.Hemo. Conc (MCHC)               | Calculated            | 34    | g/dL                   | 31.5-34.5           |
| Red Cell Distribution Width - SD (RDW-SD) | Calculated            | 42.1  | fL                     | 39-46               |
| Red Cell Distribution Width (RDW - CV)    | Calculated            | 12.8  | %                      | 11.6-14             |
| RED CELL DISTRIBUTION WIDTH INDEX (RDWI)  | Calculated            | 227.4 | -                      | *Refer Note below   |
| MENTZER INDEX                             | Calculated            | 17.8  | -                      | *Refer Note below   |
| <b>TOTAL LEUCOCYTE COUNT (WBC)</b>        | HF & FC               | 6.06  | X 10 <sup>3</sup> / μL | 4.0 - 10.0          |
| <b>DIFFERENTIAL LEUCOCYTE COUNT</b>       |                       |       |                        |                     |
| Neutrophils Percentage                    | Flow Cytometry        | 54.4  | %                      | 40-80               |
| Lymphocytes Percentage                    | Flow Cytometry        | 37.3  | %                      | 20-40               |
| Monocytes Percentage                      | Flow Cytometry        | 5.6   | %                      | 2-10                |
| Eosinophils Percentage                    | Flow Cytometry        | 1.8   | %                      | 1-6                 |
| Basophils Percentage                      | Flow Cytometry        | 0.7   | %                      | 0-2                 |
| Immature Granulocyte Percentage (IG%)     | Flow Cytometry        | 0.2   | %                      | 0-0.5               |
| Nucleated Red Blood Cells %               | Flow Cytometry        | 0.01  | %                      | 0.0-5.0             |
| <b>ABSOLUTE LEUCOCYTE COUNT</b>           |                       |       |                        |                     |
| Neutrophils - Absolute Count              | Calculated            | 3.3   | X 10 <sup>3</sup> / μL | 2.0-7.0             |
| Lymphocytes - Absolute Count              | Calculated            | 2.26  | X 10 <sup>3</sup> / μL | 1.0-3.0             |
| Monocytes - Absolute Count                | Calculated            | 0.34  | X 10 <sup>3</sup> / μL | 0.2 - 1.0           |
| Basophils - Absolute Count                | Calculated            | 0.04  | X 10 <sup>3</sup> / μL | 0.02 - 0.1          |
| Eosinophils - Absolute Count              | Calculated            | 0.11  | X 10 <sup>3</sup> / μL | 0.02 - 0.5          |
| Immature Granulocytes (IG)                | Calculated            | 0.01  | X 10 <sup>3</sup> / μL | 0-0.3               |
| Nucleated Red Blood Cells                 | Calculated            | 0.01  | X 10 <sup>3</sup> / μL | 0.0-0.5             |
| <b>PLATELET COUNT</b>                     | HF & EI               | 277   | X 10 <sup>3</sup> / μL | 150-410             |
| Mean Platelet Volume (MPV)                | Calculated            | 9.7   | fL                     | 6.5-12              |
| Platelet Distribution Width (PDW)         | Calculated            | 11.1  | fL                     | 9.6-15.2            |
| Platelet to Large Cell Ratio (PLCR)       | Calculated            | 22.5  | %                      | 19.7-42.4           |
| Plateletcrit (PCT)                        | Calculated            | 0.27  | %                      | 0.19-0.39           |

**Remarks :** Alert!!! Predominantly normocytic normochromic with ovalocytes. Platelets:Appear adequate in smear.

\*Note - Mentzer index (MI), RDW-CV and RDWI are hematological indices to differentiate between Iron Deficiency Anemia (IDA) and Beta Thalassemia Trait (BTT). MI >13, RDWI >220 and RDW-CV >14 more likely to be IDA. MI <13, RDWI <220, and RDW-CV <14 more likely to be BTT. Suggested Clinical correlation. BTT to be confirmed with HB electrophoresis if clinically indicated.

Method : Fully automated bidirectional analyser

(Reference : \*FC- flowcytometry, \*HF- hydrodynamic focussing, \*EI- Electric Impedence, \*Hb- hemoglobin, \*CPH- Cumulative pulse height)

Tests Done : HBA PROFILE,HEMOGRAM - 6 PART (DIFF)

Report Remarks : Clinically Tested by: Thyrocare Technologies Ltd

Dr Amulya MD (Path)

Dr Rajesh LG  
MD(Path)



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HYDERABAD-1

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Sample Received on (SRT) : 09 Sep 2026 11:14  
Report Released on (RRT) : 09 Sep 2026 13:57  
Sample Type | Barcode : EDTA Whole Blood | GP028145

| TEST NAME | TECHNOLOGY | VALUE | UNITS |
|-----------|------------|-------|-------|
| HbA1c     | H.P.L.C    | 4.5   | %     |

**Bio. Ref. Interval. :**

**As per ADA Guidelines**

Below 5.7% : Normal  
5.7% - 6.4% : Prediabetic  
>=6.5% : Diabetic

**Guidance For Known Diabetics**

Below 6.5% : Good Control  
6.5% - 7% : Fair Control  
7.0% - 8% : Unsatisfactory Control  
>8% : Poor Control

**Method :** Fully Automated H.P.L.C method

|                                    |                   |           |              |
|------------------------------------|-------------------|-----------|--------------|
| <b>AVERAGE BLOOD GLUCOSE (ABG)</b> | <b>CALCULATED</b> | <b>82</b> | <b>mg/dL</b> |
|------------------------------------|-------------------|-----------|--------------|

**Bio. Ref. Interval. :**

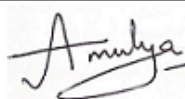
90 - 120 mg/dl : Good Control  
121 - 150 mg/dl : Fair Control  
151 - 180 mg/dl : Unsatisfactory Control  
> 180 mg/dl : Poor Control

**Method :** Derived from HBA1c values

**Please correlate with clinical conditions.**

Tests Done : HBA PROFILE,HEMOGRAM - 6 PART (DIFF)

Report Remarks : Clinically Tested by: Thyrocare Technologies Ltd-NABL  
accredited, NABL certificate no: MC-5241



Dr Amulya MD (Path)



Dr Abdur R MD  
(Biochem)



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HYDERABAD-1

Sample Collected on (SCT) : 09 Sep 2026 06:53  
Sample Received on (SRT) : 09 Sep 2026 11:15  
Report Released on (RRT) : 09 Sep 2026 13:02  
Sample Type | Barcode : FLUORIDE PLASMA | GN843844

| TEST NAME                    | TECHNOLOGY | VALUE | UNITS |
|------------------------------|------------|-------|-------|
| FASTING BLOOD SUGAR(GLUCOSE) | PHOTOMETRY | 87    | mg/dL |

**Bio. Ref. Interval. :-**

| As per ADA Guideline: Fasting Plasma Glucose (FPG) |                        |
|----------------------------------------------------|------------------------|
| Normal                                             | 70 to 100 mg/dl        |
| Prediabetes                                        | 100 mg/dl to 125 mg/dl |
| Diabetes                                           | 126 mg/dl or higher    |

**Note :**

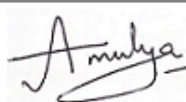
The assay could be affected mildly and may result in anomalous values if serum samples have heterophilic antibodies, hemolyzed , icteric or lipemic. The concentration of Glucose in a given specimen may vary due to differences in assay methods, calibration and reagent specificity. For diagnostic purposes results should always be assessed in conjunction with patients medical history, clinical findings and other findings.

**Please correlate with clinical conditions.**

**Method:-** GOD-POD METHOD

Tests Done : BLOOD SUGAR (F)

Report Remarks : Clinically Tested by: Thyrocare Technologies Ltd-NABL  
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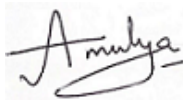
Sample Collected on (SCT) : 09 Sep 2026 06:53  
Sample Received on (SRT) : 09 Sep 2026 11:16  
Report Released on (RRT) : 09 Sep 2026 14:34  
Sample Type | Barcode : URINE | GR488291

| TEST NAME                             | METHODOLOGY           | VALUE                    | UNITS        | Bio. Ref. Interval. |
|---------------------------------------|-----------------------|--------------------------|--------------|---------------------|
| <b>Complete Urinogram</b>             |                       |                          |              |                     |
| <b><u>Physical Examination</u></b>    |                       |                          |              |                     |
| VOLUME                                | Visual Determination  | 3                        | mL           | -                   |
| COLOUR                                | Visual Determination  | PALE YELLOW              | -            | Pale Yellow         |
| APPEARANCE                            | Visual Determination  | CLEAR                    | -            | Clear               |
| SPECIFIC GRAVITY                      | pKa change            | 1.015                    | -            | 1.003-1.030         |
| PH                                    | pH indicator          | 6.5                      | -            | 5-8                 |
| <b><u>Chemical Examination</u></b>    |                       |                          |              |                     |
| URINARY PROTEIN                       | PEI                   | ABSENT                   | mg/dL        | Absent              |
| URINARY GLUCOSE                       | GOD-POD               | ABSENT                   | mg/dL        | Absent              |
| URINE KETONE                          | Nitroprusside         | ABSENT                   | mg/dL        | Absent              |
| URINARY BILIRUBIN                     | Diazo coupling        | ABSENT                   | mg/dL        | Absent              |
| <b>UROBILINOGEN</b>                   | <b>Diazo coupling</b> | <b>Trace(1-2 mg/dl )</b> | <b>mg/dL</b> | <b>&lt;=0.2</b>     |
| BILE SALT                             | Hays sulphur          | ABSENT                   | -            | Absent              |
| BILE PIGMENT                          | Ehrlich reaction      | ABSENT                   | -            | Absent              |
| URINE BLOOD                           | Peroxidase reaction   | ABSENT                   | -            | Absent              |
| NITRITE                               | Diazo coupling        | ABSENT                   | -            | Absent              |
| LEUCOCYTE ESTERASE                    | Esterase reaction     | ABSENT                   | -            | Absent              |
| <b><u>Microscopic Examination</u></b> |                       |                          |              |                     |
| MUCUS                                 | Microscopy            | ABSENT                   | -            | Absent              |
| RED BLOOD CELLS                       | Microscopy            | 1                        | cells/HPF    | 0-2                 |
| URINARY LEUCOCYTES (PUS CELLS)        | Microscopy            | ABSENT                   | cells/HPF    | 0-5                 |
| EPITHELIAL CELLS                      | Microscopy            | 3                        | cells/HPF    | 0-5                 |
| CASTS                                 | Microscopy            | ABSENT                   | -            | Absent              |
| CRYSTALS                              | Microscopy            | ABSENT                   | -            | Absent              |
| BACTERIA                              | Microscopy            | ABSENT                   | -            | Absent              |
| YEAST                                 | Microscopy            | ABSENT                   | -            | Absent              |
| PARASITE                              | Microscopy            | ABSENT                   | -            | Absent              |

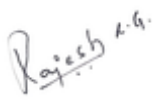
(Reference : \*PEI - Protein error of indicator, \*GOD-POD - Glucose oxidase-peroxidase)

Tests Done : COMPLETE URINE ANALYSIS

Report Remarks : Clinically Tested by: Thyrocare Technologies Ltd



Dr Amulya MD (Path)



Dr Rajesh LG  
MD(Path)



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Referred By : SELF  
Address : 6-7-63 RAJU COLONY HINDUSTAN AERONAUTICS HYD  
HYDERABAD-1

Sample Collected on (SCT) : 09 Sep 2026 06:53  
Sample Received on (SRT) : 09 Sep 2026 11:37  
Report Released on (RRT) : 10 Sep 2026 07:50  
Sample Type | Barcode : SERUM | GM537954

| TEST NAME                 | TECHNOLOGY | VALUE  | UNITS | Bio. Ref. Interval |
|---------------------------|------------|--------|-------|--------------------|
| TOTAL CHOLESTEROL         | PHOTOMETRY | 182    | mg/dL | < 200              |
| HDL CHOLESTEROL - DIRECT  | PHOTOMETRY | 30     | mg/dL | 40-60              |
| LDL CHOLESTEROL - DIRECT  | PHOTOMETRY | 127.05 | mg/dL | < 100              |
| TRIGLYCERIDES             | PHOTOMETRY | 162    | mg/dL | < 150              |
| TC/ HDL CHOLESTEROL RATIO | CALCULATED | 6      | Ratio | 3 - 5              |
| TRIG / HDL RATIO          | CALCULATED | 5.31   | Ratio | < 3.12             |
| LDL / HDL RATIO           | CALCULATED | 4.2    | Ratio | 1.5-3.5            |
| HDL / LDL RATIO           | CALCULATED | 0.24   | Ratio | > 0.40             |
| NON-HDL CHOLESTEROL       | CALCULATED | 151.96 | mg/dL | < 160              |
| VLDL CHOLESTEROL          | CALCULATED | 32.34  | mg/dL | 5 - 40             |

Please correlate with clinical conditions.

#### Method :

CHOL - Cholesterol Oxidase, Esterase, Peroxidase  
HCHO - Direct Enzymatic Colorimetric  
LDL - Direct Measure  
TRIG - Enzymatic, End Point  
TC/H - Derived from serum Cholesterol and Hdl values  
TRI/H - Derived from TRIG and HDL Values  
LDL/ - Derived from serum HDL and LDL Values  
HD/LD - Derived from HDL and LDL values.  
NHDH - Derived from serum Cholesterol and HDL values  
VLDL - Derived from serum Triglyceride values

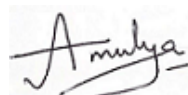
#### \*REFERENCE RANGES AS PER NCEP ATP III GUIDELINES:

| TOTAL CHOLESTEROL | (mg/dl) | HDL  | (mg/dl) | LDL             | (mg/dl) | TRIGLYCERIDES   | (mg/dl) |
|-------------------|---------|------|---------|-----------------|---------|-----------------|---------|
| DESIRABLE         | <200    | LOW  | <40     | OPTIMAL         | <100    | NORMAL          | <150    |
| BORDERLINE HIGH   | 200-239 | HIGH | >60     | NEAR OPTIMAL    | 100-129 | BORDERLINE HIGH | 150-199 |
| HIGH              | >240    |      |         | BORDERLINE HIGH | 130-159 | HIGH            | 200-499 |
|                   |         |      |         | HIGH            | 160-189 | VERY HIGH       | >500    |
|                   |         |      |         | VERY HIGH       | >190    |                 |         |

Alert !!! 10-12 hours fasting is mandatory for lipid parameters. If not, values might fluctuate.

Tests Done : Comprehensive Full Body Checkup With Vitamin D and  
B12 - 3,IRON DEFICIENCY PROFILE  
ADVANCED,MC,TESTOSTERONE

Report Remarks : Clinically Tested by: Thyrocare Technologies Ltd-NABL  
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Sample Type | Barcode : SERUM | GM537954

| TEST NAME                                                       | TECHNOLOGY       | VALUE       | UNITS        |
|-----------------------------------------------------------------|------------------|-------------|--------------|
| <b>25-OH VITAMIN D (TOTAL)</b><br><b>Bio. Ref. Interval. :-</b> | <b>E.C.L.I.A</b> | <b>18.9</b> | <b>ng/mL</b> |

Deficiency :  $\leq 20$  ng/ml || Insufficiency : 21-29 ng/ml  
Sufficiency :  $\geq 30$  ng/ml || Toxicity :  $> 100$  ng/ml

Clinical Significance:

Vitamin D is a fat soluble vitamin that has been known to help the body absorb and retain calcium and phosphorous; both are critical for building bone health.

Decrease in vitamin D total levels indicate inadequate exposure of sunlight, dietary deficiency, nephrotic syndrome.

Increase in vitamin D total levels indicate Vitamin D intoxication.

Specifications: Precision: Intra assay (%CV):9.20%, Inter assay (%CV):8.50%

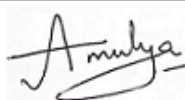
Kit Validation Reference : Holick M. Vitamin D the underappreciated D-Lightful hormone that is important for Skeletal and cellular health Curr Opin Endocrinol Diabetes 2002;9(1)87-98.

**Please correlate with clinical conditions.**

**Method:-** Fully Automated Electrochemiluminescence Competitive Immunoassay

Tests Done : Comprehensive Full Body Checkup With Vitamin D and  
B12 - 3, IRON DEFICIENCY PROFILE  
ADVANCED, MG, TESTOSTERONE

Report Remarks : Clinically Tested by: Thyrocare Technologies Ltd-NABL  
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| TEST NAME                                            | TECHNOLOGY       | VALUE      | UNITS        |
|------------------------------------------------------|------------------|------------|--------------|
| <b>VITAMIN B-12</b><br><b>Bio. Ref. Interval. :-</b> | <b>E.C.L.I.A</b> | <b>153</b> | <b>pg/mL</b> |

Normal: 197-771 pg/ml

Clinical significance :

Vitamin B12 or cyanocobalamin, is a complex corrinoid compound found exclusively from animal dietary sources, such as meat, eggs and milk. It is critical in normal DNA synthesis, which in turn affects erythrocyte maturation and in the formation of myelin sheath. Vitamin-B12 is used to find out neurological abnormalities and impaired DNA synthesis associated with macrocytic anemias. For diagnostic purpose, results should always be assessed in conjunction with the patients medical history, clinical examination and other findings.

Specifications: Intra assay (%CV):2.6%, Inter assay (%CV):2.3 %

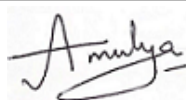
Kit Validation Reference : Thomas L.Clinical laborator Diagnostics : Use and Assessment of Clinical laboratory Results 1st Edition,TH Books-Verl-Ges,1998:424-431

**Please correlate with clinical conditions.**

**Method:-** Fully Automated Electrochemiluminescence Compititive Immunoassay

Tests Done : Comprehensive Full Body Checkup With Vitamin D and  
B12 - 3,IRON DEFICIENCY PROFILE  
ADVANCED,MG,TESTOSTERONE

Report Remarks : Clinically Tested by: Thyrocare Technologies Ltd-NABL  
accredited, NABL certificate no: MC-5241



Dr Amulya MD (Path)



Dr Abdur R MD  
(Biochem)



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A wing, 'Dattani Plaza', Near East West Industrial Estate, Safed Pool, Saki Naka,  
Andheri (East), Mumbai - 400072

Patient Name : SRINATH(24Y/M)  
Referred By : SELF  
Address : 6-7-63 RAJU COLONY HINDUSTAN AERONAUTICS HYD  
HYDERABAD-1

Sample Collected on (SCT) : 09 Sep 2026 06:53  
Sample Received on (SRT) : 09 Sep 2026 11:37  
Report Released on (RRT) : 10 Sep 2026 07:50  
Sample Type | Barcode : SERUM | GM537954

| TEST NAME                   | TECHNOLOGY | VALUE | UNITS  | Bio. Ref. Interval. |
|-----------------------------|------------|-------|--------|---------------------|
| TOTAL TRIIODOTHYRONINE (T3) | E.C.L.I.A  | 92    | ng/dL  | 80-200              |
| TOTAL THYROXINE (T4)        | E.C.L.I.A  | 6.1   | µg/dL  | 4.8-12.7            |
| TSH - ULTRASENSITIVE        | E.C.L.I.A  | 2.71  | µIU/mL | 0.54-5.30           |

Comments : \*\*\*

The Biological Reference Ranges is specific to the age group. Kindly correlate clinically.

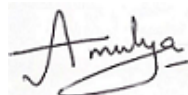
**Method :**

T3,T4 - Fully Automated Electrochemiluminescence Compititive Immunoassay  
USTSH - Fully Automated Electrochemiluminescence Sandwich Immunoassay

**Disclaimer :**Results should always be interpreted using the reference range provided by the laboratory that performed the test. Different laboratories do tests using different technologies, methods and using different reagents which may cause difference. In reference ranges and hence it is recommended to interpret result with assay specific reference ranges provided in the reports. To diagnose and monitor therapy doses, it is recommended to get tested every time at the same Laboratory.

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| TEST NAME                  | TECHNOLOGY | VALUE | UNITS | Bio. Ref. Interval |
|----------------------------|------------|-------|-------|--------------------|
| BLOOD UREA NITROGEN (BUN)  | PHOTOMETRY | 11.82 | mg/dL | 7.94 - 20.07       |
| CREATININE - SERUM         | PHOTOMETRY | 0.89  | mg/dL | 0.72-1.18          |
| BUN / Sr.CREATININE RATIO  | CALCULATED | 13.28 | Ratio | 9:1-23:1           |
| UREA (CALCULATED)          | CALCULATED | 25.29 | mg/dL | Adult : 17-43      |
| UREA / SR.CREATININE RATIO | CALCULATED | 28.42 | Ratio | < 52               |
| CALCIUM                    | PHOTOMETRY | 9.57  | mg/dL | 8.8-10.6           |
| URIC ACID                  | PHOTOMETRY | 6     | mg/dL | 4.2 - 7.3          |

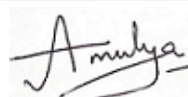
**Please correlate with clinical conditions.**

**Method :**

BUN - Kinetic UV Assay.  
SCRE - Creatinine Enzymatic Method  
B/CR - Derived from serum Bun and Creatinine values  
UREAC - Derived from BUN Value.  
UR/CR - Derived from UREA and Sr.Creatinine values.  
CALC - Arsenazo III Method, End Point.  
URIC - Uricase / Peroxidase Method

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Report Remarks : Clinically Tested by: Thyrocare Technologies Ltd



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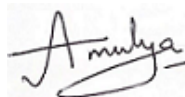
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Sample Type | Barcode : SERUM | GM537954

| TEST NAME                                                                                                                                                                          | TECHNOLOGY | VALUE | UNITS |
|------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|------------|-------|-------|
| IRON<br><b>Bio. Ref. Interval. :</b><br>Male : 65 - 175<br>Female : 50 - 170<br><b>Method :</b> Ferrozine method without deproteinization                                          | PHOTOMETRY | 82    | µg/dL |
| TOTAL IRON BINDING CAPACITY (TIBC)<br><b>Bio. Ref. Interval. :</b><br>Male: 225 - 535 µg/dl Female: 215 - 535 µg/dl<br><b>Method :</b> Spectrophotometric Assay                    | PHOTOMETRY | 398   | µg/dL |
| % TRANSFERRIN SATURATION<br><b>Bio. Ref. Interval. :</b><br>13 - 45<br><b>Method :</b> Derived from IRON and TIBC values                                                           | CALCULATED | 21    | %     |
| FERRITIN<br><b>Bio. Ref. Interval. :</b><br>Men: 21.81 - 274.66 ng/ml<br>Women: 4.63 - 204.00 ng/ml<br><b>Method :</b> Fully Automated Chemi Luminescent Microparticle Immunoassay | C.M.I.A    | 106.1 | ng/mL |
| UNSAT.IRON-BINDING CAPACITY(UIBC)<br><b>Bio. Ref. Interval. :</b><br>162 - 368<br><b>Method :</b> SPECTROPHOTOMETRIC ASSAY                                                         | PHOTOMETRY | 315.5 | µg/dL |

**Please correlate with clinical conditions.**

Tests Done : Comprehensive Full Body Checkup With Vitamin D and  
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| TEST NAME                                          | TECHNOLOGY | VALUE | UNITS |
|----------------------------------------------------|------------|-------|-------|
| TRANSFERRIN SERUM<br><b>Bio. Ref. Interval. :-</b> | Photometry | 315   | mg/dL |

Adults : 200-360 mg/dL  
Children (0-4 days): 130-275 mg/dL  
Children 3 Months-10 yrs : 203-360 mg/dL

CLINICAL SIGNIFICANCE:

MEASUREMENT OF TRANSFERRIN LEVELS IN SERUM AIDS IN THE DIAGNOSIS OF MALNUTRITION, ACUTE INFLAMMATION INFECTION AND RED BLOOD CELL DISORDERS SUCH AS DEFICIENCY ANEMIA.

SPECIFICATION:

PRECISION %CV : 1.19% , SENSITIVITY : 0.4 mg/dL

KIT VALIDATION REFERENCE:

Grant GH, Silverman LM, Christenson RH. Amino Acids and proteins. In: Tietz NW, ed. Fundamentals of clinical chemistry. Philadelphia: WB Saunders Company, 1987:333-334pp.

**Please correlate with clinical conditions.**

**Method:-** Immunoturbidimetry

Tests Done : Comprehensive Full Body Checkup With Vitamin D and  
B12 - 3,IRON DEFICIENCY PROFILE  
ADVANCED,MG,TESTOSTERONE

Report Remarks : Clinically Tested by: Thyrocare Technologies Ltd-NABL  
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Dr Ranjana Roy  
MD(Path)



Dr Shruty Sonar  
MD(Path)



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| TEST NAME                              | TECHNOLOGY | VALUE | UNITS |
|----------------------------------------|------------|-------|-------|
| TESTOSTERONE<br>Bio. Ref. Interval. :- | E.C.L.I.A  | 285   | ng/dL |

280 - 800

Clinical Significance: Clinical evaluation of serum testosterone, along with serum LH, assists in evaluation of Hypogonadal males. Major causes of lowered testosterone in males include Hypogonadotropic hypogonadism, testicular failure Hyperprolactinemia, Hypopituitarism some types of liver and kidney diseases and critical illness.

Specifications: Precision: Intra assay (%CV): 11.50 %, Inter assay (%CV): 5.70%; Sensitivity: 7 ng/dL.  
Kit Validation Reference: Wilson JD Foster DW (Eds) Williams Textbook of Endocrinology 8th Edition WB Saunders Piladelphia Pennsylvania.

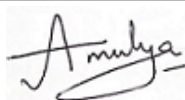
Note : The Biological Reference Range mentioned is specific to the age group and gender. Kindly correlate clinically.

**Please correlate with clinical conditions.**

**Method:-** Fully Automated Electrochemiluminescence Compititive Immunoassay

Tests Done : Comprehensive Full Body Checkup With Vitamin D and  
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(Biochem)



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| TEST NAME                          | TECHNOLOGY        | VALUE       | UNITS        | Bio. Ref. Interval |
|------------------------------------|-------------------|-------------|--------------|--------------------|
| ALKALINE PHOSPHATASE               | PHOTOMETRY        | 68.21       | U/L          | 45-129             |
| BILIRUBIN - TOTAL                  | PHOTOMETRY        | 0.48        | mg/dL        | 0.3-1.2            |
| BILIRUBIN -DIRECT                  | PHOTOMETRY        | 0.09        | mg/dL        | 0 - 0.20           |
| BILIRUBIN (INDIRECT)               | CALCULATED        | 0.39        | mg/dL        | 0-0.9              |
| GAMMA GLUTAMYL TRANSFERASE (GGT)   | PHOTOMETRY        | 17.67       | U/L          | < 55               |
| ASPARTATE AMINOTRANSFERASE (SGOT ) | PHOTOMETRY        | 19.13       | U/L          | < 35               |
| ALANINE TRANSAMINASE (SGPT)        | PHOTOMETRY        | 27.9        | U/L          | < 45               |
| SGOT / SGPT RATIO                  | CALCULATED        | 0.69        | Ratio        | < 2                |
| PROTEIN - TOTAL                    | PHOTOMETRY        | 7.59        | gm/dL        | 5.7-8.2            |
| <b>ALBUMIN - SERUM</b>             | <b>PHOTOMETRY</b> | <b>4.84</b> | <b>gm/dL</b> | <b>3.2-4.8</b>     |
| SERUM GLOBULIN                     | CALCULATED        | 2.75        | gm/dL        | 2.5-3.4            |
| SERUM ALB/GLOBULIN RATIO           | CALCULATED        | 1.76        | Ratio        | 0.9 - 2            |

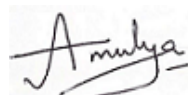
**Please correlate with clinical conditions.**

**Method :**

ALKP - Modified IFCC method  
BILT - Diazonium salt DPD method  
BILD - Diazonium salt DPD method  
BILI - Derived from serum Total and Direct Bilirubin values  
GGT - Modified IFCC method  
SGOT - IFCC\* Without Pyridoxal Phosphate Activation  
SGPT - IFCC\* Without Pyridoxal Phosphate Activation  
OT/PT - Derived from SGOT and SGPT values.  
PROT - Biuret Method  
SALB - Albumin Bcg<sup>1</sup>method (Colorimetric Assay Endpoint)  
SEGB - DERIVED FROM SERUM ALBUMIN AND PROTEIN VALUES  
A/GR - Derived from serum Albumin and Protein values

Tests Done : Comprehensive Full Body Checkup With Vitamin D and  
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Dr Abdur R MD  
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| TEST NAME | TECHNOLOGY | VALUE | UNITS |
|-----------|------------|-------|-------|
| MAGNESIUM | PHOTOMETRY | 2.09  | mg/dL |

**Bio. Ref. Interval. :-**

1.90 - 3.10 mg/dL

**Clinical significance:**

Magnesium is the fourth most abundant cation in the body and second most prevalent intracellular cation. The total body magnesium content is about 25 g or approximately 1 mol, of which 55% reside in the skeleton. About 45% of the magnesium is intracellular. In general higher the metabolic activity of cell, the greater is its magnesium content. Magnesium is a cofactor for more than 300 enzymes in the body.

Disorders of magnesium metabolism are separated into those causing hypomagnesaemia/magnesium deficiencies and hypermagnesemia. Hypomagnesaemia is common in patient in hospitals. Moderate to severe deficiency of magnesium is usually due to loss of magnesium from the gastrointestinal (gi) tract or kidneys. One of the more serious complications of magnesium deficiency is cardiac arrhythmia. Symptomatic hypermagnesemia is almost always caused by excessive intake, resulting from administration of antacids, enemas, and parenteral fluids containing magnesium. Depression of neuromuscular system is the most common manifestation of magnesium intoxication.

**External quality control program participation:**

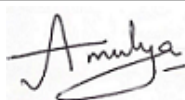
College Of American Pathologists: Chemistry survey; CAP Number: 7193855-01

**Please correlate with clinical conditions.**

**Method:-** MODIFIED XYLIDYL BLUE REACTION METHOD

Tests Done : Comprehensive Full Body Checkup With Vitamin D and  
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Sample Type | Barcode : SERUM | GM537954

| TEST NAME                                                               | TECHNOLOGY | VALUE | UNITS          |
|-------------------------------------------------------------------------|------------|-------|----------------|
| EST. GLOMERULAR FILTRATION RATE (eGFR)<br><b>Bio. Ref. Interval. :-</b> | CALCULATED | 123   | mL/min/1.73 m2 |

> = 90 : Normal  
60 - 89 : Mild Decrease  
45 - 59 : Mild to Moderate Decrease  
30 - 44 : Moderate to Severe Decrease  
15 - 29 : Severe Decrease  
<15 : Kidney Failure

#### Clinical Significance

The normal serum creatinine reference interval does not necessarily reflect a normal GFR for a patient. Because mild and moderate kidney injury is poorly inferred from serum creatinine alone. Thus, it is recommended for clinical laboratories to routinely estimate glomerular filtration rate (eGFR), a "gold standard" measurement for assessment of renal function, and report the value when serum creatinine is measured for patients 18 and older, when appropriate and feasible. It cannot be measured easily in clinical practice, instead, GFR is estimated from equations using serum creatinine, age, race and sex. This provides easy to interpret information for the doctor and patient on the degree of renal impairment since it approximately equates to the percentage of kidney function remaining. Application of CKD-EPI equation together with the other diagnostic tools in renal medicine will further improve the detection and management of patients with CKD.

#### Reference

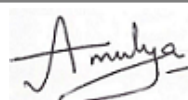
Levey AS, Stevens LA, Schmid CH, Zhang YL, Castro AF, 3rd, Feldman HI, et al. A new equation to estimate glomerular filtration rate. Ann Intern Med. 2009;150(9):604-12.

#### Please correlate with clinical conditions.

**Method:-** 2021 CKD EPI Creatinine Equation

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#### CONDITIONS OF REPORTING

- ✓ The reported results are for information and interpretation of the referring doctor only.
- ✓ It is presumed that the tests performed on the specimen belong to the patient; named or identified.
- ✓ Results of tests may vary from laboratory to laboratory and also in some parameters from time to time for the same patient.
- ✓ Should the results indicate an unexpected abnormality, the same should be reconfirmed.
- ✓ Only such medical professionals who understand reporting units, reference ranges and limitations of technologies should interpret results.
- ✓ This report is not valid for medico-legal purpose.
- ✓ Docon Technologies Private Limited, Thyrocare Technologies Limited and its employees/representatives do not assume any liability, responsibility for any loss or damage that may be incurred by any person as a result of presuming the meaning or contents of the report.

#### EXPLANATIONS

- ✓ **Name** - The name is as declared by the client and recorded by the personnel who collected the specimen.
- ✓ **Ref.By** - The name of the doctor who has recommended testing as declared by the client.
- ✓ **Labcode** - This is the accession number in our laboratory and it helps us in archiving and retrieving the data.
- ✓ **Barcode** - This is the specimen identity number and it states that the results are for the specimen bearing the barcode (irrespective of the name).
- ✓ **SCT** - Specimen Collection Time - The time when specimen was collected as declared by the client.
- ✓ **SRT** - Specimen Receiving Time - This time when the specimen reached our laboratory.
- ✓ **RRT** - Report Releasing Time - The time when our pathologist has released the values for Reporting.
- ✓ **Reference Range** - Means the range of values in which 95% of the normal population would fall.

#### SUGGESTIONS

- ✓ Values out of reference range requires reconfirmation before starting any medical treatment.
- ✓ Retesting is needed if you suspect any quality shortcomings.
- ✓ For suggestions, complaints or feedback, write to us at [grievance-office@docon.co.in](mailto:grievance-office@docon.co.in) or call us on 7022000900.

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